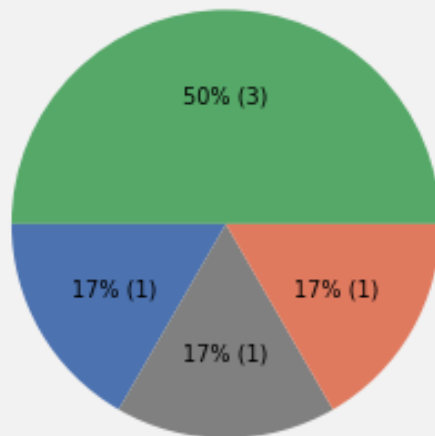


Aldia Hong

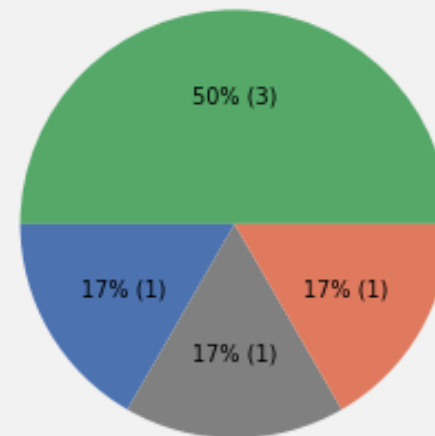
Number of forms: 6

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- Completed with minimal assistance
- Completed independently (competent)
- No basis for evaluation

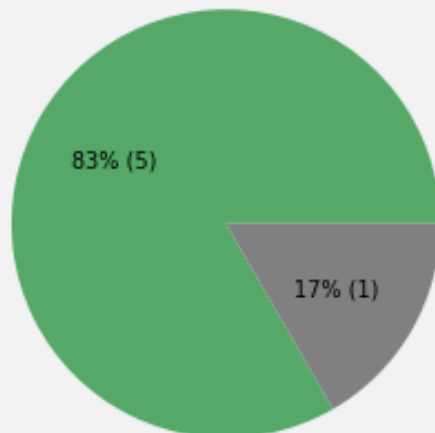
1.1 Obtain and interpret a prenatal, natal and neonatal history, medical history, dental history and social history and identify the chief complaint



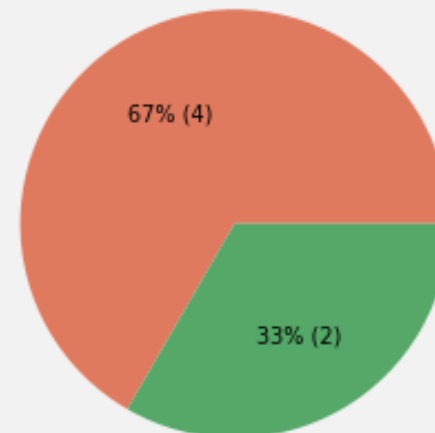
1.2 Conduct a thorough and accurate clinical examination



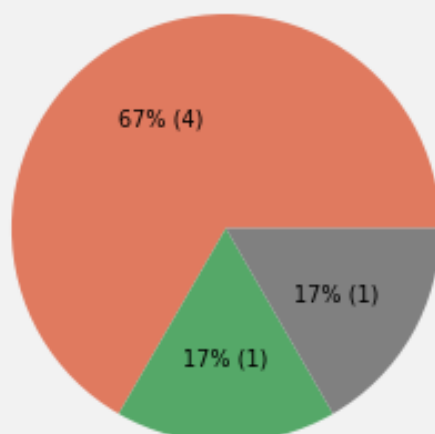
1.3 Determine the appropriate radiographic study and obtain diagnostic radiographs with minimal radiation exposure to patient and personnel



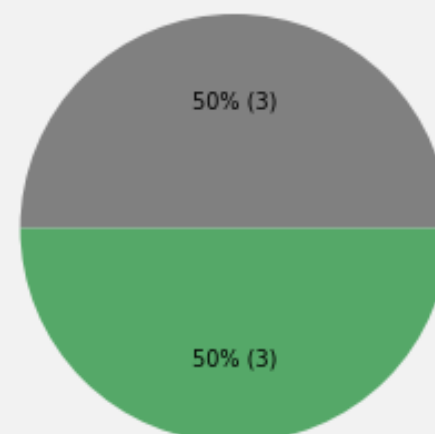
1.4 Interpret findings from history, clinical exam and diagnostic tests, and arrive at a diagnosis

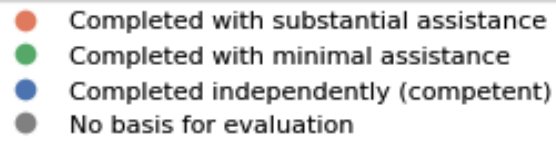


1.5 Develop a comprehensive, logically sequenced, individualized treatment plan based on the diagnosis

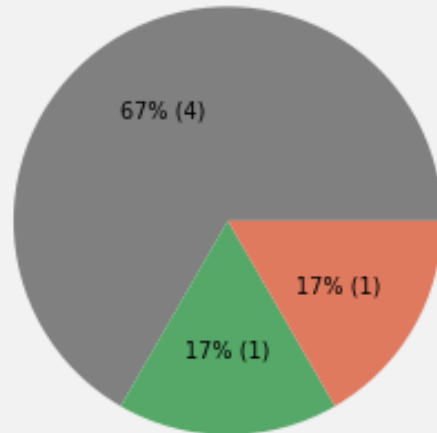


1.6 Recognize when consultation with other clinician(s) is indicated and provide a written referral(s) that includes appropriate diagnostic material

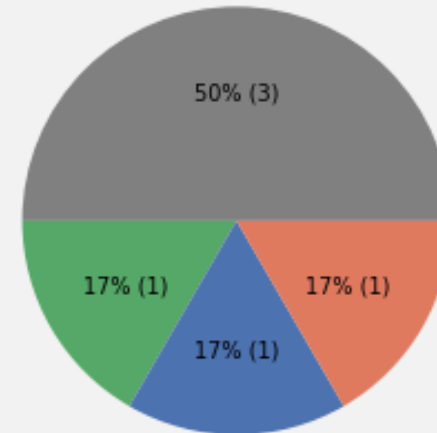




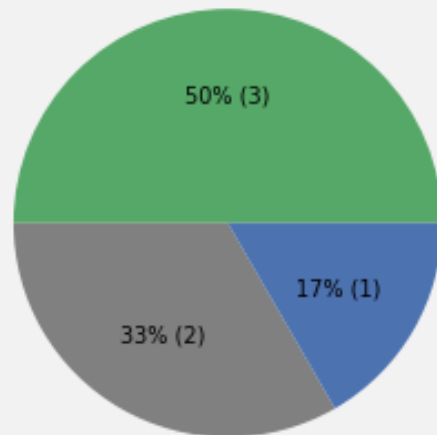
1.7 Recognize the significance of special health care needs in individual patients and formulate a treatment plan that utilizes the alternatives in the delivery of dental care that those conditions may require



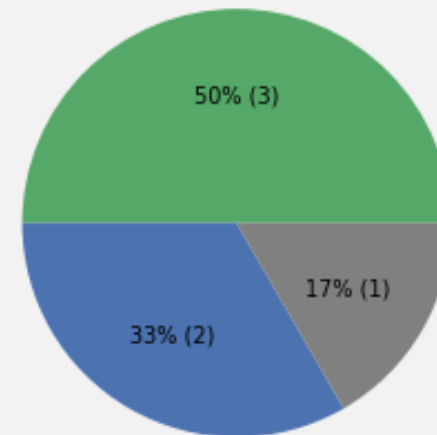
1.8 Explain and discuss the findings, diagnosis and treatment options with the patient, parent and /or guardian including an explanation of a treatment plan that has been modified due to medical or physical limitations, behavioral, financial, or other unexpected factors



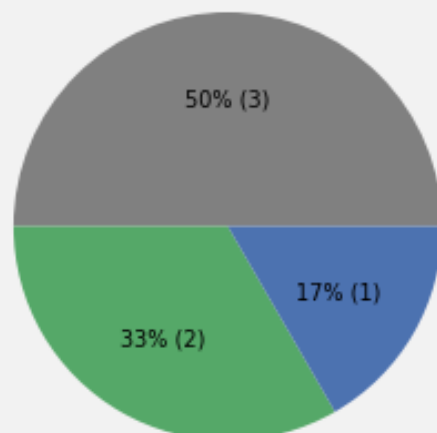
1.9 Discuss financial obligations and obtain informed consent from the legally responsible party



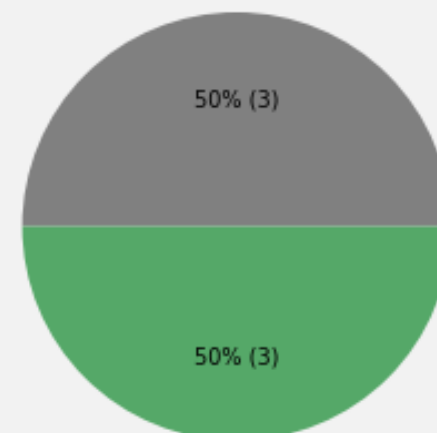
1.10 Establish and maintain accurate patient treatment records



2.1 Provide developmentally and age-appropriate education to patients, parents and caretakers concerning the etiology and scientific principals underlying the prevention of oral diseases including diet management, chemotherapeutics and other approaches

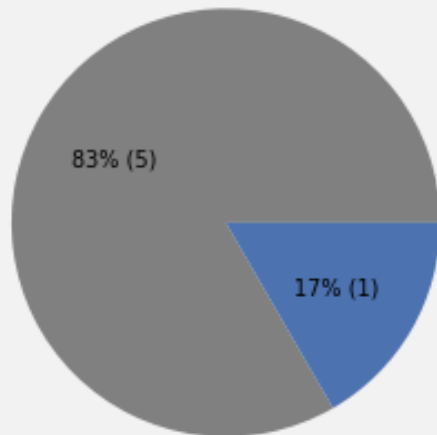


2.2 Implement individualized plans for health promotion and disease prevention by identifying factors that may impede behavioral change and compliance, and implement strategies to motivate patients, parents and caretakers to assume responsibility for their oral health

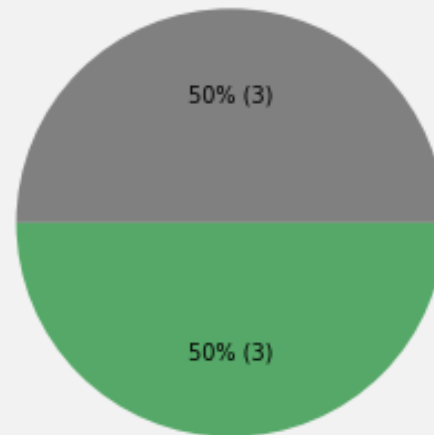


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- No basis for evaluation

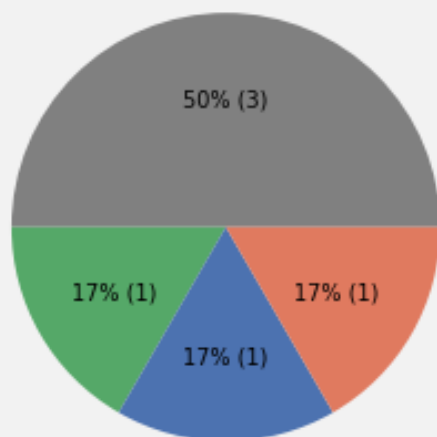
2.3 Evaluate the effectiveness of preventive programs and make adjustments when indicated



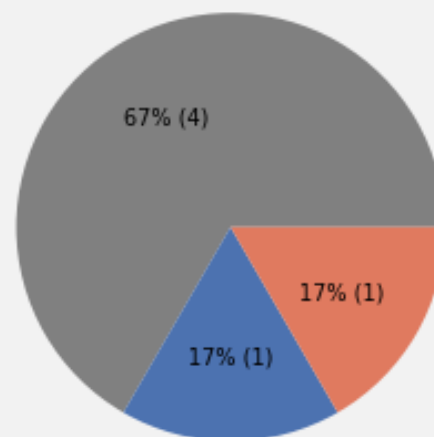
3.1 Perform a differential diagnosis of patient fear and anxiety and select the appropriate behavior guidance technique to alleviate pain and anxiety



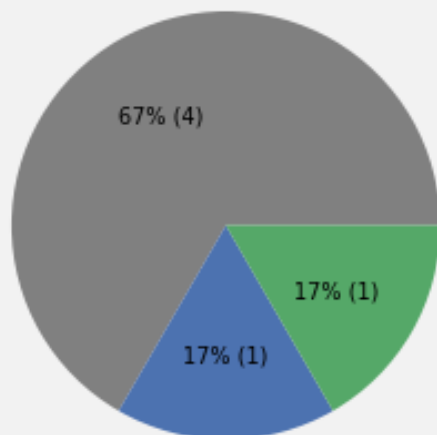
3.2 Describe to parents, in understandable terms, the various behavior guidance techniques and obtain informed consent



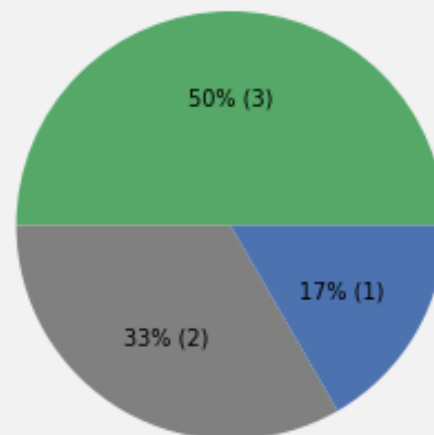
3.3 Apply age-appropriate and culturally sensitive non-pharmacologic behavior guidance techniques effectively in an out-patient setting



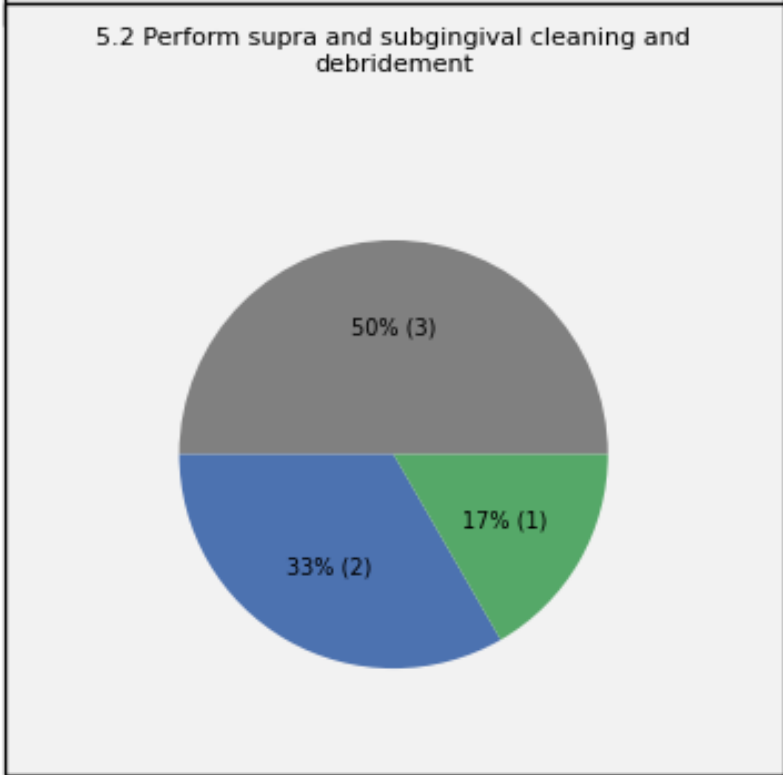
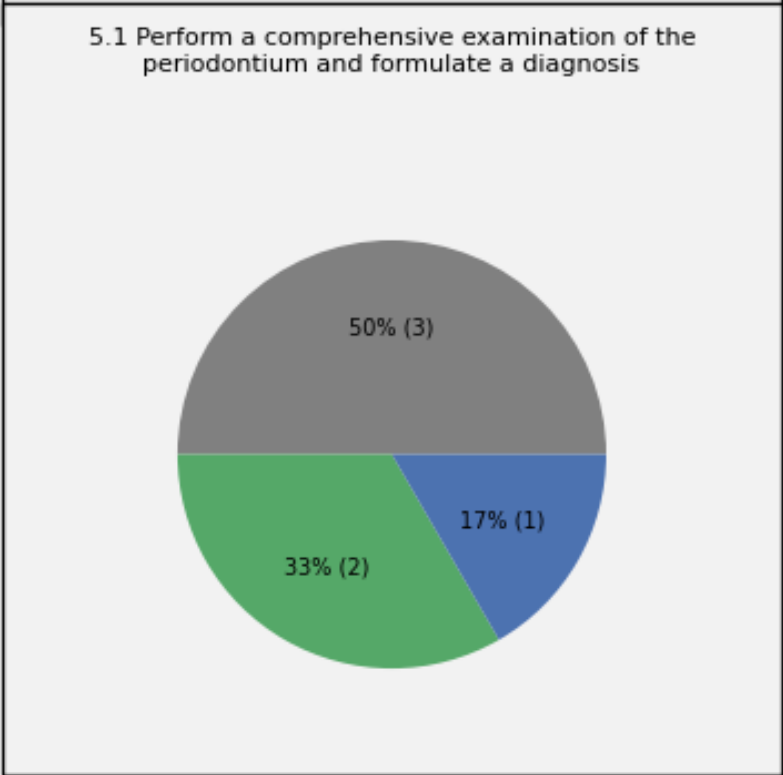
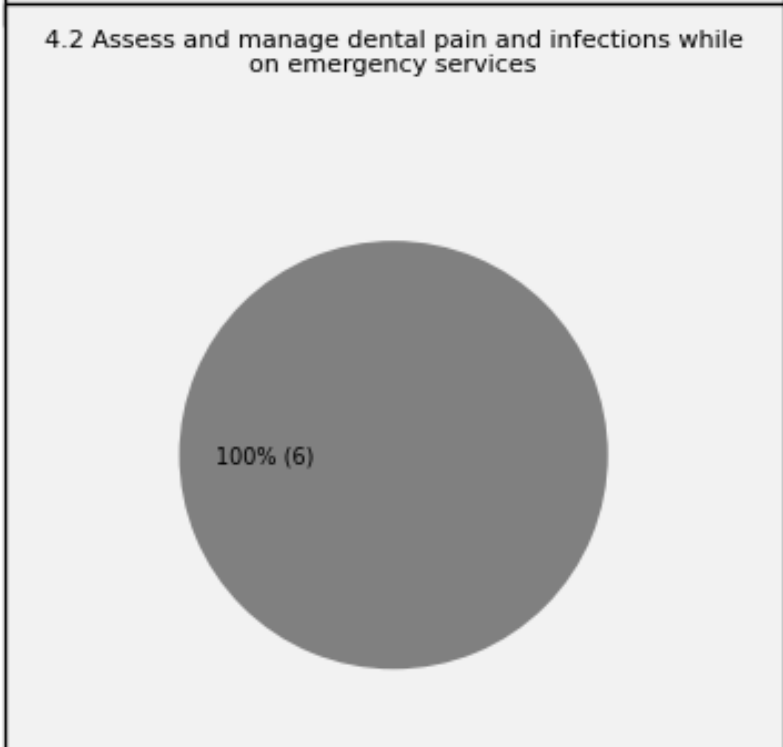
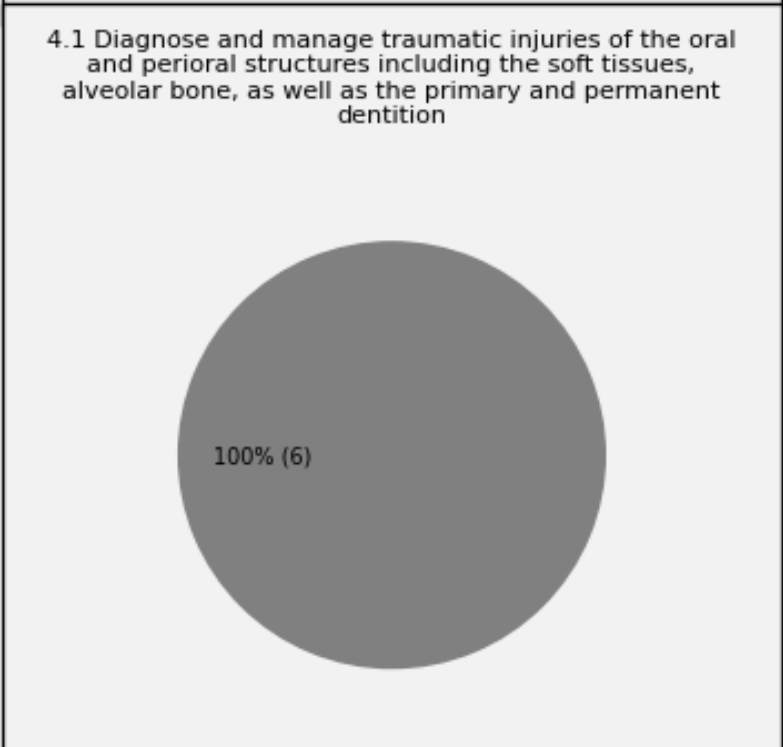
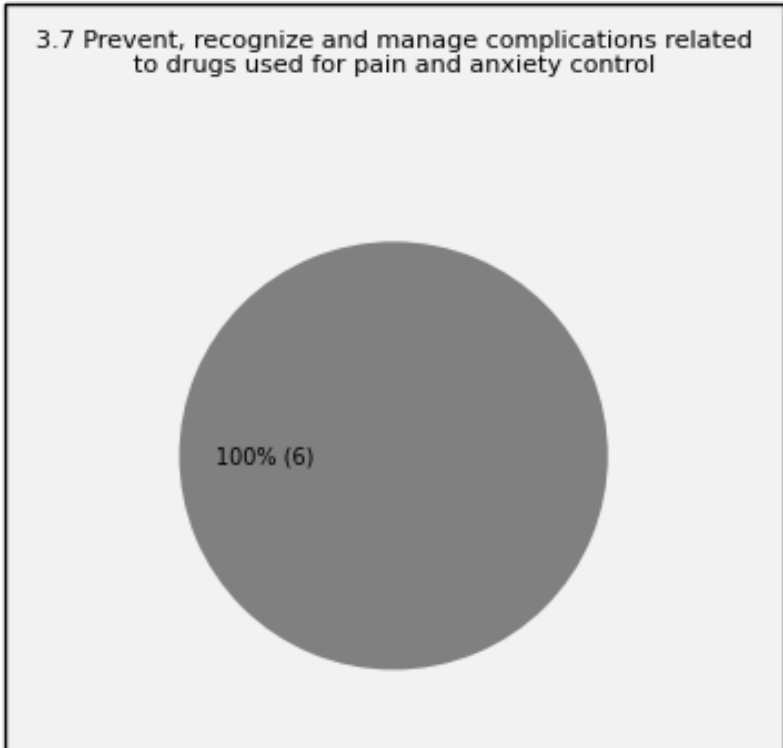
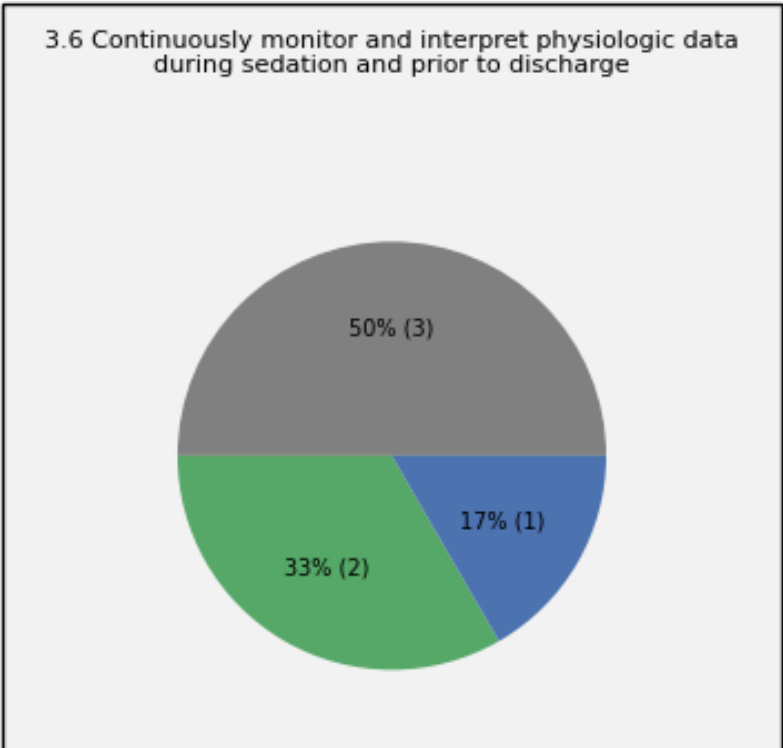
3.4 Utilize nitrous oxide inhalation appropriately and effectively



3.5 Perform pre-operative workup and assessment including medical risk assessment prior to sedation and general anesthesia

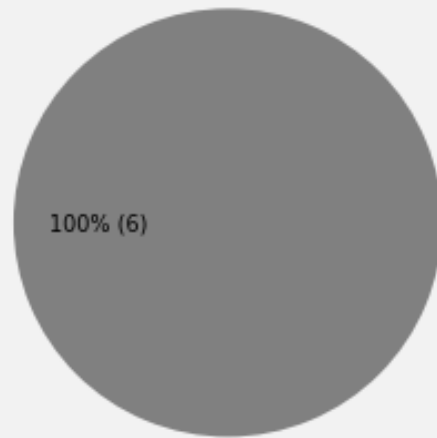


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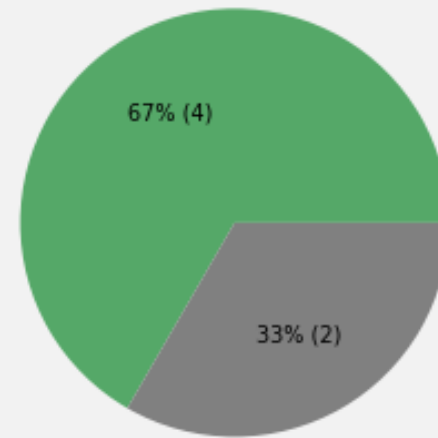


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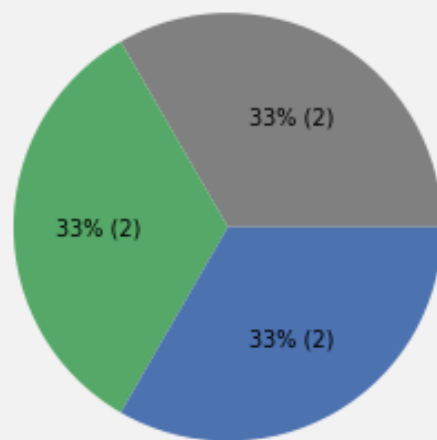
5.3 Recognize when specialist periodontal care is indicated and make an appropriate referral



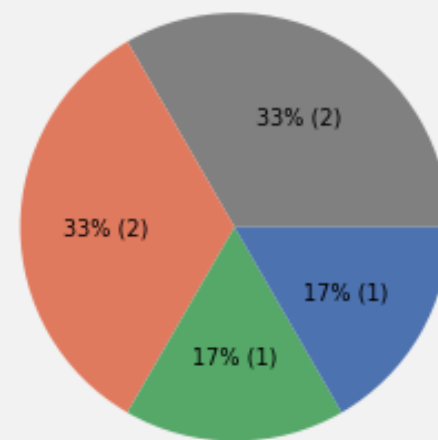
6.1 Understand the properties and indications for contemporary restorative and prosthetic techniques available for the restoration of primary, mixed and permanent dentition



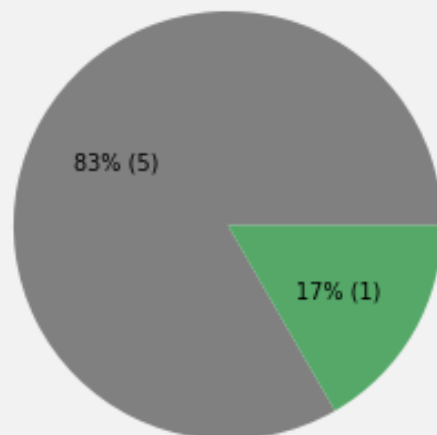
6.2 Administer local anesthesia safely and effectively, and appropriately manage local and general complications



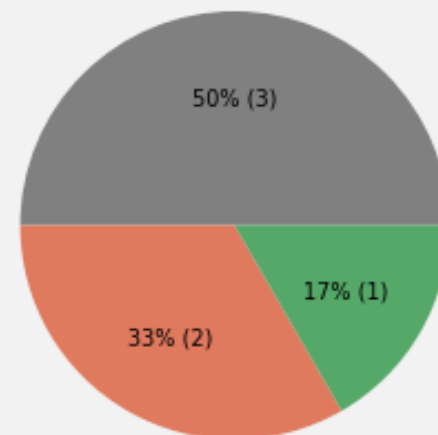
6.3 Utilize the various restorative and prosthetic techniques appropriately and effectively, and be able to analyze failures



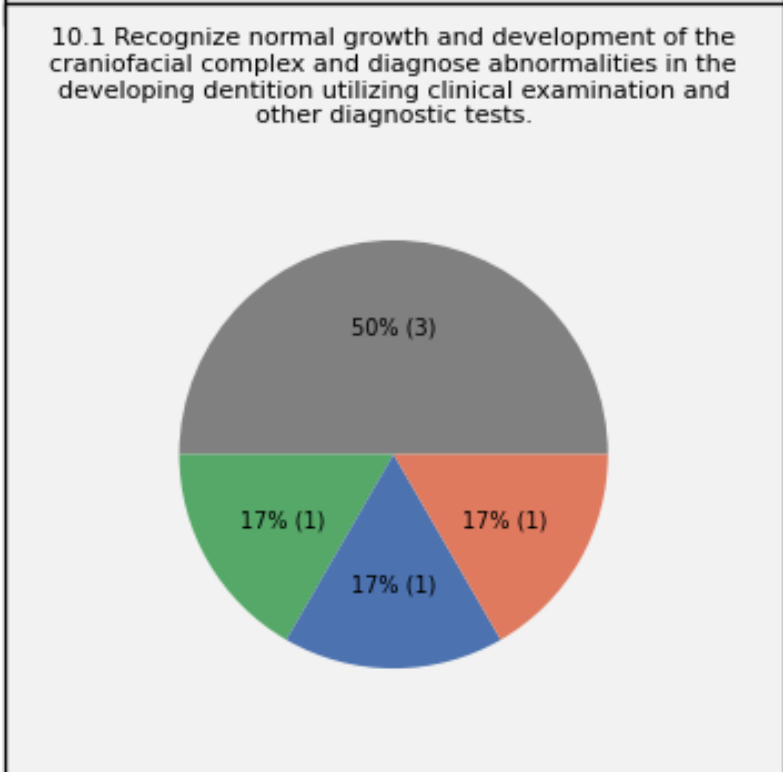
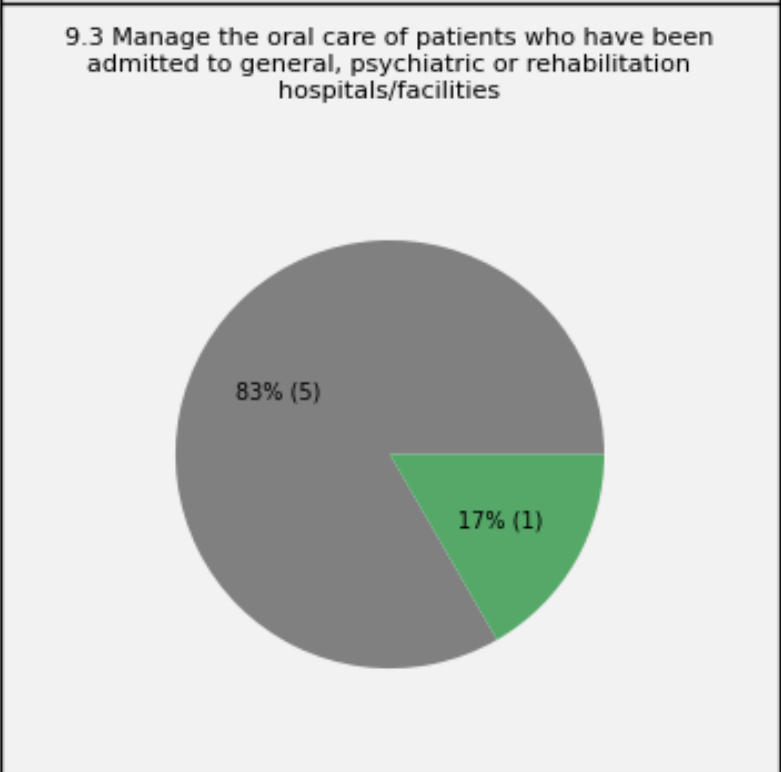
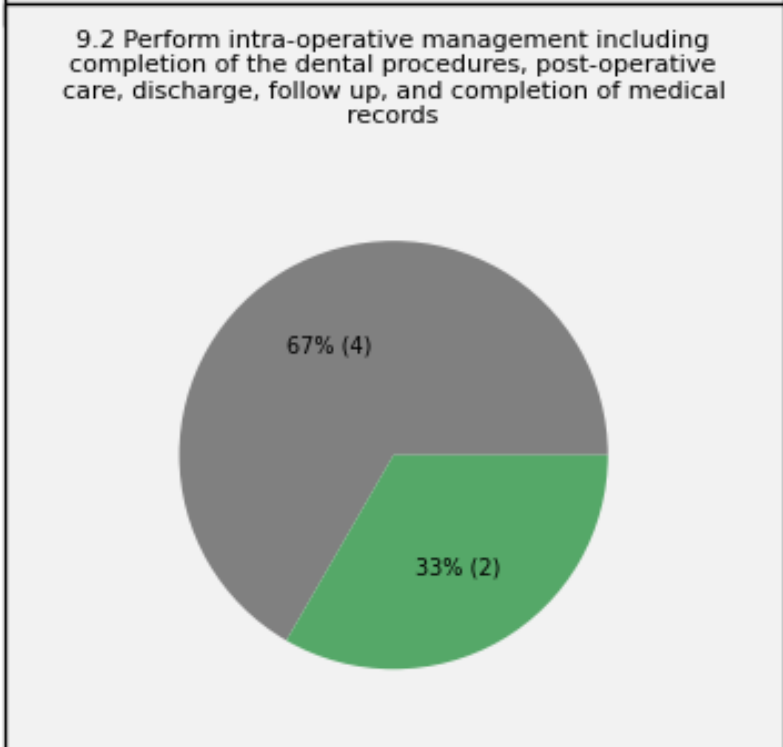
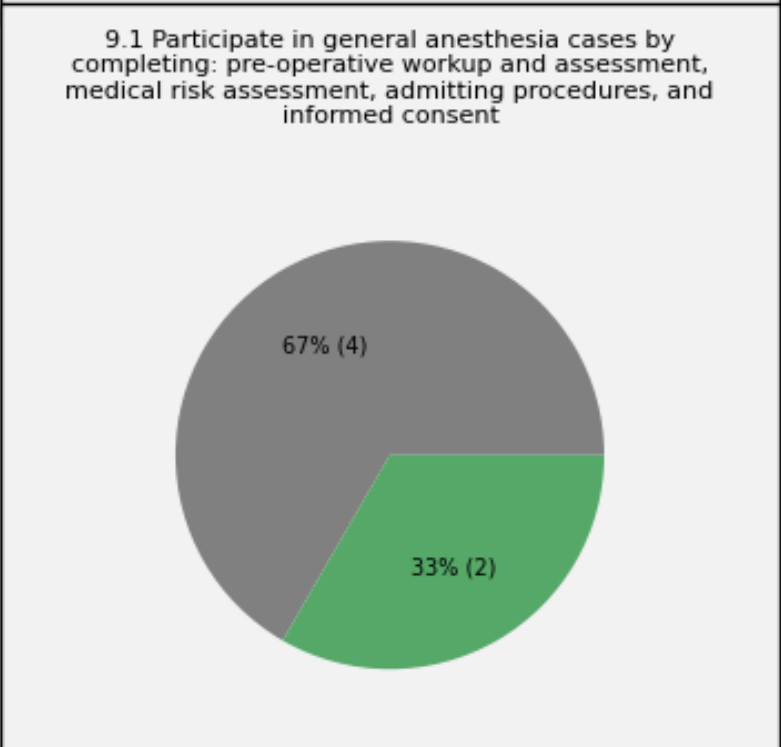
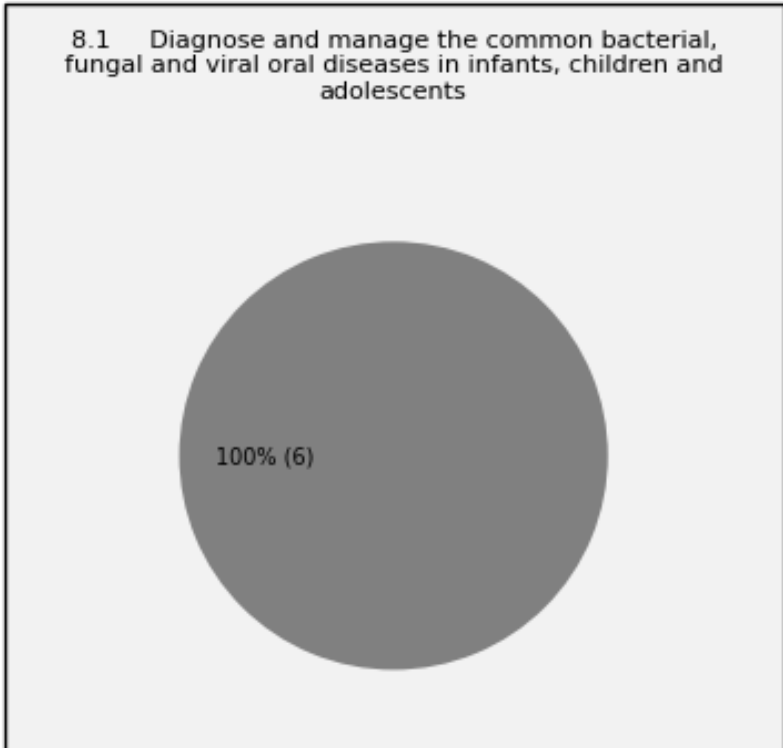
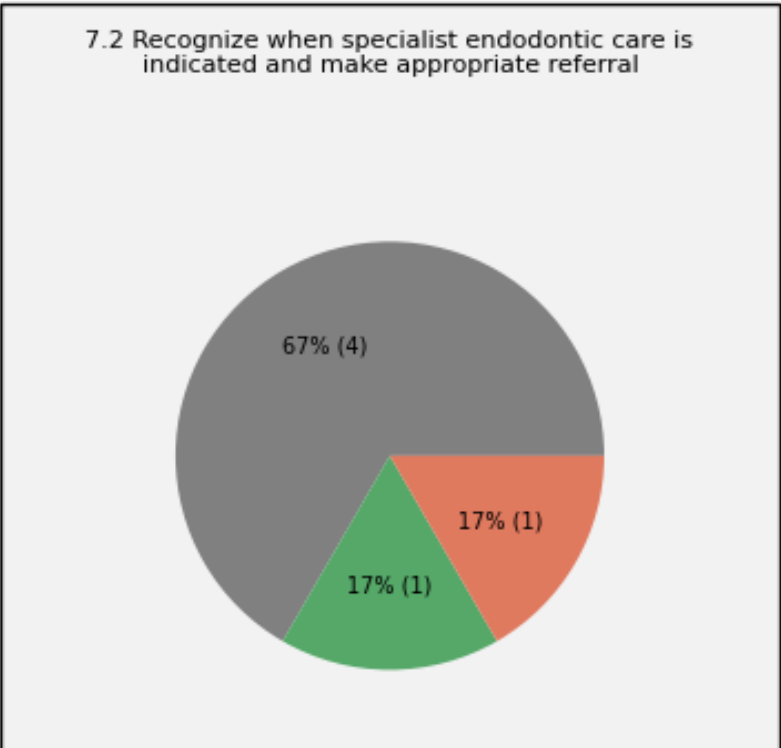
6.4 Lead multi-disciplinary teams in providing comprehensive care for healthy patients as well as those who are medically, physically, mentally or socially compromised



7.1 Diagnose and manage pulpal disease in primary as well as immature and mature permanent teeth

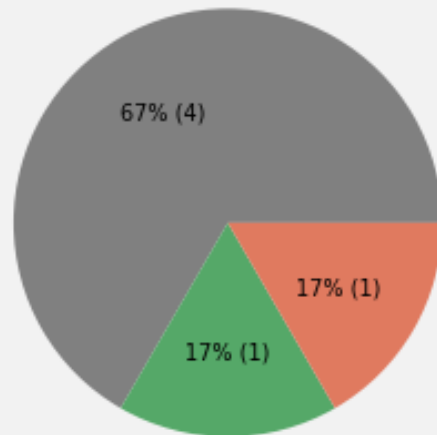


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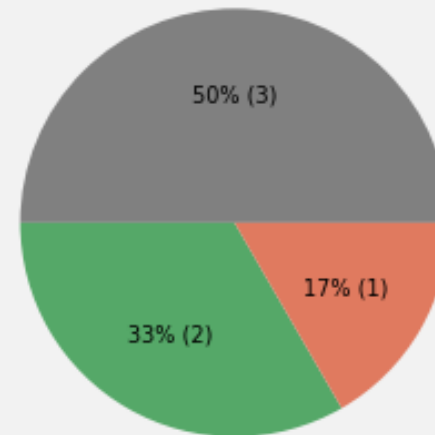


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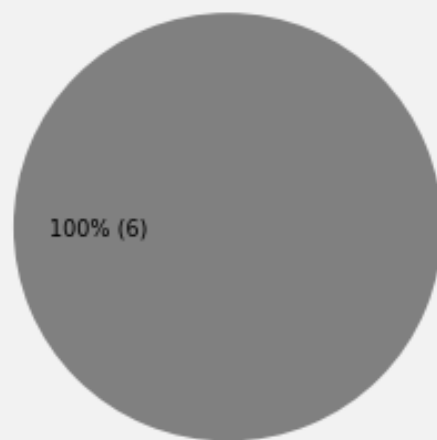
10.2 Recognize the indications, contraindications and fundamental treatment modalities used in guidance of eruption and space supervision of the developing dentition.



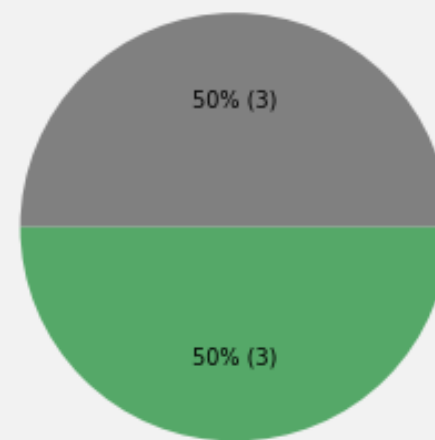
10.3 Recognize when consultation with and/or timely referral to orthodontic or other specialist care is indicated in order to achieve optimal outcomes in the developing occlusion



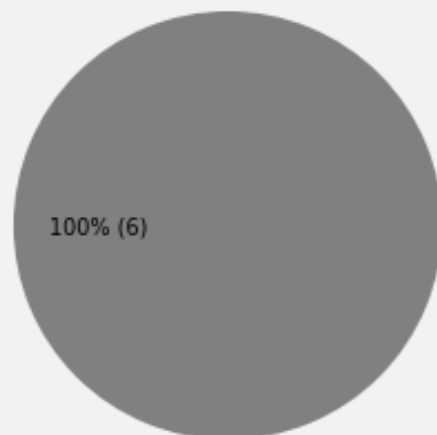
10.4 Perform interceptive removable and/or fixed orthodontic procedures in the primary, mixed, and early permanent dentition to establish a stable, functional and esthetic occlusion



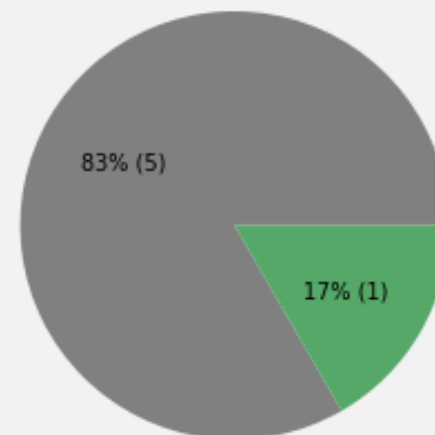
11.1 Perform extraction of uncomplicated primary and permanent teeth including fractured or residual root tips



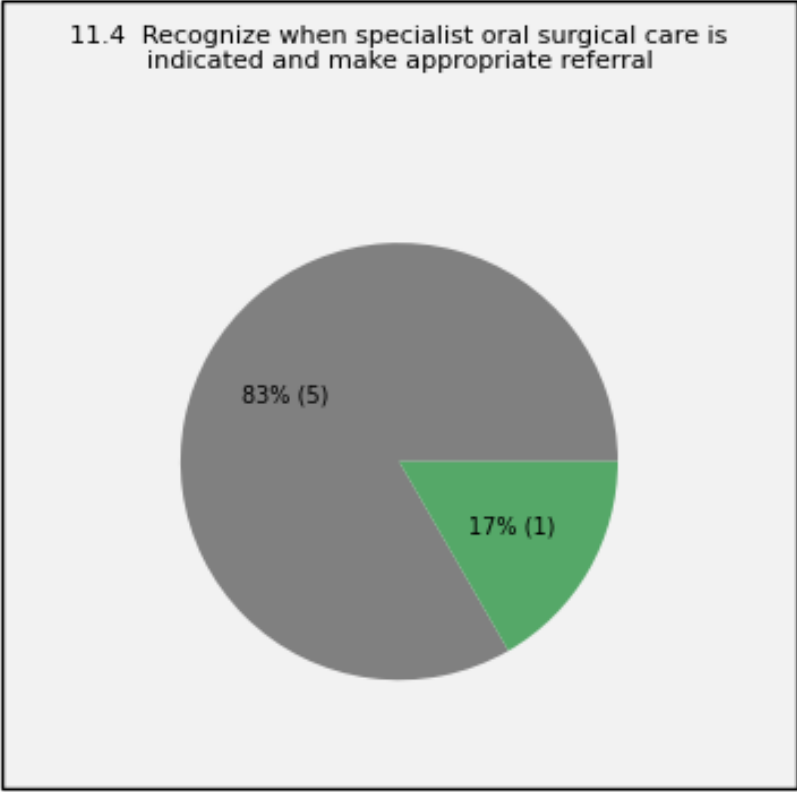
11.2 Perform minor oral surgical procedures including excisional and incisional biopsy, soft tissue surgery and exposure of impacted teeth



11.3 Recognize and manage common intra-operative and post-operative surgical complications

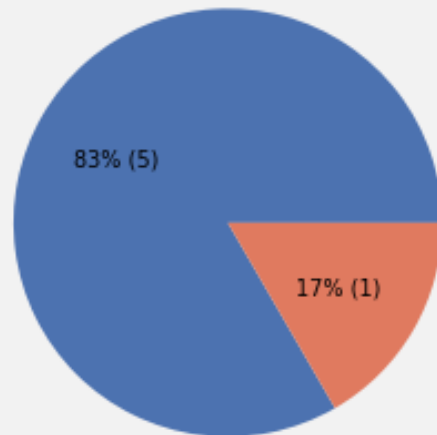


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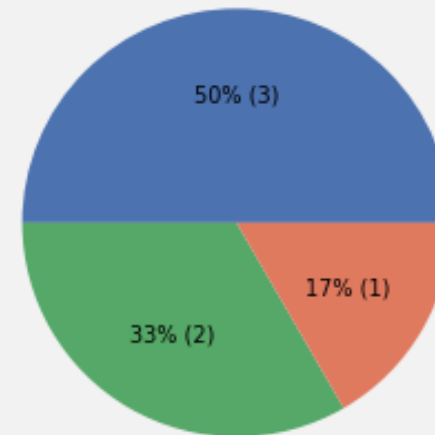


- ALWAYS
- OFTEN
- SOMETIMES
- RARELY
- NEVER
- Not graded

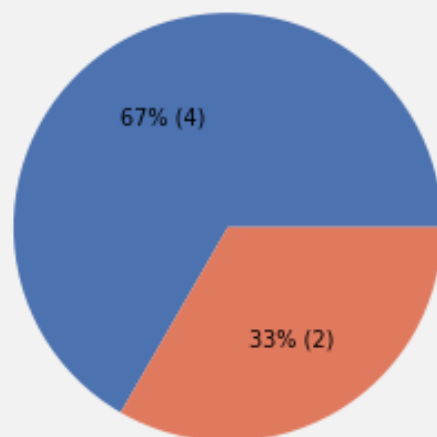
12.1 Appropriate professional attire and appearance



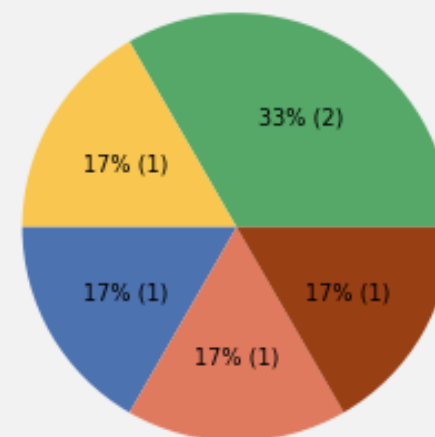
12.2 Punctual and prepared for clinic



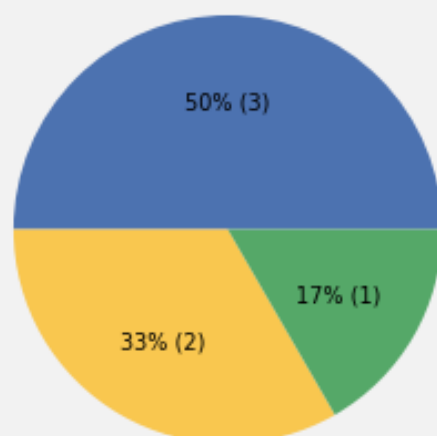
12.3 Speaks respectfully to parents and patients



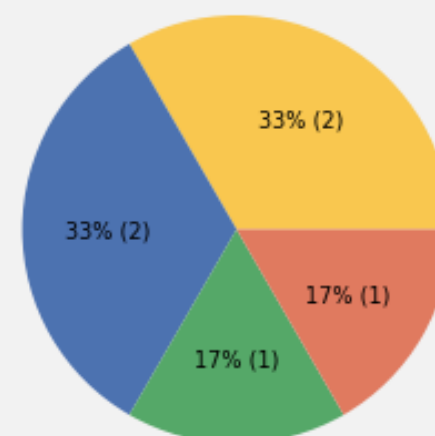
12.4 Self-evaluates clinical performance



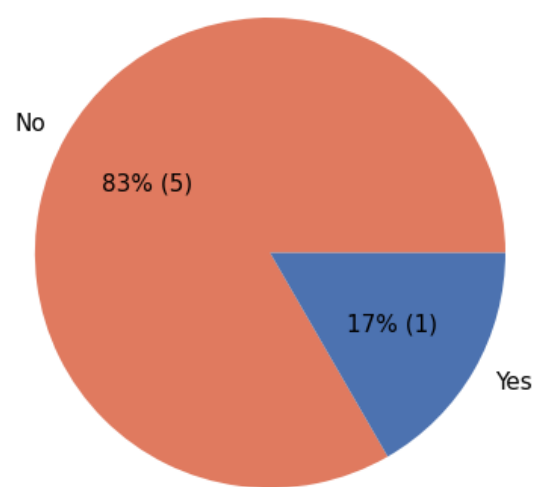
12.5 Seeks appropriate faculty advice



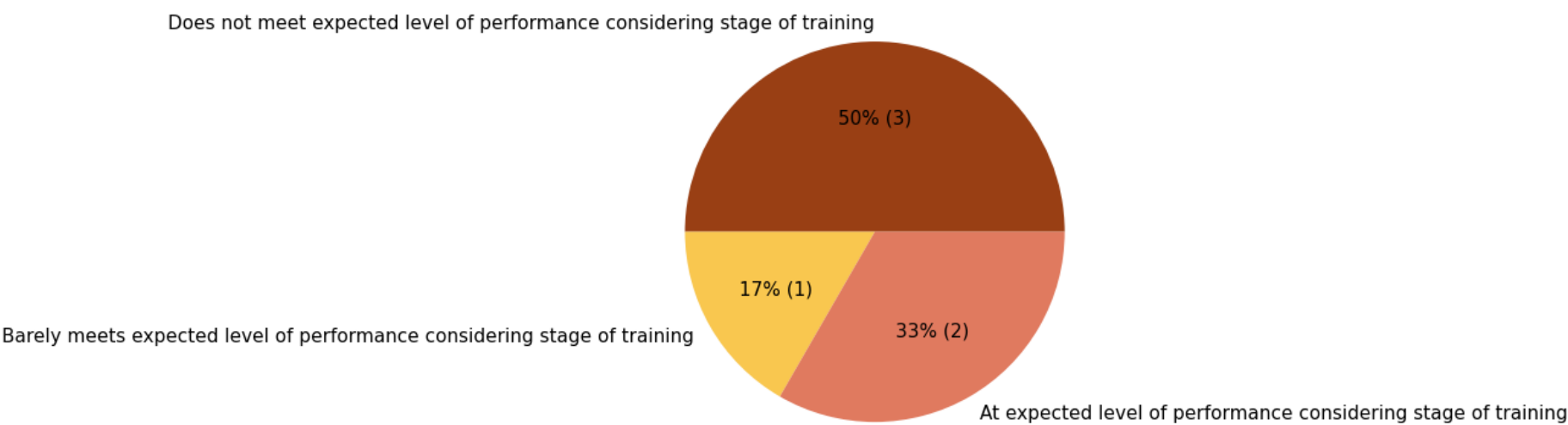
12.6 Alters behaviour based on faculty advice



Competent to work semi-independently at Royal Children's Hospital



Final Grades



Describe the strongest aspects of the student's performance, including examples of noteworthy incidents

Date	Staff	Comments
2025-11-06	Nicky Kilpatrick	I need to note that I have not supervised Aldia very much this semester so am not in a good position to comment.
2025-11-06	Heshani Kellapatha	Improving clinical and tx planning skills
2025-11-07	Steph Shields	Aldia has improved over the year and is more prepared for clinic and more methodical in her presenting and treatment planning
2025-11-08	Dharini Ravindra	Aldia is enthusiastic and eager to learn, she would benefit from slowing down and thinking through each step before completing it.
2025-11-10	Daniel Andreasen-Cocker	Note some entries are on this form as there is not a NA for 19. For prepared for clinic I have given this mark because she hasn't been prepared for case discussion. For Q18 I have concerns based on 2 case discussions.
2025-11-18	Wendy Cheney	Dr. Hong has made progress in some areas of her clinical work and struggled in other aspects. Her patient presentations are more thorough and logical than earlier in the year when she frequently missed crucial diagnoses.

Describe any aspect of the student's performance where he/she needs to spend more time and effort. Include any examples of critical incidents

Date	Staff	Comments
2025-11-06	Nicky Kilpatrick	I worry that Aldia does not have good insight in to what is required/expected of her. She would benefit from considering her communication style and working on how to come across more professionally to inspire trust and confidence.
2025-11-06	Heshani Kellapatha	-
2025-11-07	Steph Shields	Aldia can have knowledge gaps that present in fairly straightforward scenarios, which may be due to her relative clinical inexperience on commencing DCD. An example is being unfamiliar with how to manage trauma having just completed a semester of didactic trauma content. She will need to spend additional time preparing for clinic and learning/revising in order to improve
2025-11-08	Dharini Ravindra	Aldia still understanding the mechanics of retention of stainless steel crowns on teeth and fitting the correct sizing. We have discussed treatment planning and spending time evaluating radiographs including the furcation areas before finalising a treatment plan. We have also discussed the importance of accuracy with crown preps and avoiding iatrogenic damage.
2025-11-10	Daniel Andreasen-Cocker	Note: This feedback is from two case discussions and no clinical observation. Aldia was rostered with me once but due to the complexity of surgical cases swapped with another student so she could focus on SSC experience which Aldia said she needed more experience with. I think Aldia needs significant work on the key skill to observe take in and synthesise data and formulate a treatment plan. Please put this feedback within the context of other consultants feedback as it is based on two treatment planning discussions. In both discussions it was clear Aldia was not putting forward her interpretation or opinions but the opinion of other consultants. This was particularly evident because she did not understand the reasoning for the recommendations, at this stage of training she will need to listen and take on the advice but the concern was she did not understand it (or could not show she could understand it) and was just regurgitating what they had said. She should be able to say I agree with Dr Wendy's opinion because xyz, rather than because Dr Wendy said so. In one example Aldia came to discuss a potential DI case. The diagnosis was not an easy one to make and the child may or may not have DI, and we have to acknowledge this at Aldia's stage of training. It was of great concern that Aldia was not prepared for the discussion not having a level of understanding I would expect a first year to have to come and have an informed and educational discussion, it was evident Aldia had not done basic reading of the subject to be able to have an informed discussion and learn from discussion. I'm not sure if this is because Aldia has not yet covered DI and OI in seminars but this would be a poor excuse and Aldia needs to know that she needs to be learning across all of paed dentistry throughout the program with self direction in additional to the seminar topics. I was particularly concerned that given the DI history there was no screening for OI, although I was concerned about the blue sclera in the photos presented and this may not have been real but due to shadowing/screen colours etc., Aldia showed a lack of understanding that there could be more than DI going on. Aldia's response to this is the child did not come with the OI diagnosis, she was lacking the understanding that a paediatric dentist can be the first person to make significant medical diagnosis and it is important to be open to the possibilities and not be solely focused on the teeth. It also concerns me greatly for on call at RCH because as a consultant on the phone you cannot check the examination is accurate and rely on the accuracy of the examination the student presents to you. From this interaction I do not have faith that Aldia would pick up on the less obvious presenting but potentially more serious issues. The second case I am concerned over Aldia's radiographic interpretation skills and ability to place that in context of a potential diagnosis and the treatment history. The teenager may or may not need RCT on one incisor and a repeat RCT of an MTA obdured tooth on the other incisor, both of which are significant calls given the need for GA and the MTA obdured tooth was only completed a few months earlier by an Endodontic specialist in training under GA. The special needs teenager may or may not need a GA, which is significant effort for the child and family. Aldia was not able to discuss potential treatment options, she could only present the one plan. However there was an option of we have no S+S except a radiograph and therefore there is an option of wait and take another in a shorter recall timeframe to review for more information before jumping to a diagnosis and treatment plan given the long term effect on prognosis of such treatment.
2025-11-18	Wendy Cheney	Dr. Hong has difficulty prioritising items from her problem list and then formulating appropriate treatment options, let alone give an evidence-based rationale. At times it is obvious she has only a basic understanding of the issue at hand and simply looks for answers from me. Her technical work is OK - but she does not routinely use rubber dam. Dr. Hong tends to work too independently - I highly recommend she seek advice from the supervisors more often. I have discussed with Dr. Hong that at times her demeanour is unprofessional - that she uses excessive body and hand movements that are quite distracting. She often appears nervous and does not make a point of sitting still and giving the parent/caretaker her full attention. Dr. Hong will need to consciously work on this aspect of her work.